

Discharge Summary

Patient:	Herbert Schröder
Date of Birth:	08/30/1968
Admission Date:	03/31/2025
Discharge Date:	04/14/2025
Attending Physician:	Dr. Eva Bergmann, MD

Mr. Herbert Schröder, DOB 08/30/1968, was voluntarily admitted on 03/31/2025 following a severe depressive episode with passive suicidal ideation. He had been on sertraline 100 mg for 8 months with inadequate response. His outpatient psychiatrist referred him for inpatient care after he disclosed stockpiling tablets and having a plan to overdose, though with no immediate intent.

Primary Diagnosis: Recurrent depressive disorder, current episode severe (F33.2)

Past Psychiatric History: Two prior depressive episodes — first at age 34 following marital breakdown, second at age 41 during occupational burnout. Both resolved with antidepressant treatment. No prior hospitalisation. No mania, psychosis, substance misuse, or prior suicide attempts.

Social History: Secondary school mathematics teacher, on sick leave for 6 weeks. Lives with partner of 12 years who attended the admission interview. Two adult children from first marriage. Partner is the primary source of social support.

Mental State on Admission: Unkempt, poor eye contact, reduced speech. Profoundly low mood, pervasive anhedonia, hopelessness, passive suicidal ideation with formed plan (medication stockpile) but no immediate intent. No psychotic features. Cognition grossly intact. Preserved insight.

Hospital Course: Sertraline increased to 150 mg day 2. Mirtazapine 15 mg added at bedtime day 3, uptitrated to 30 mg day 7. CBT-based individual therapy 3 sessions per week from day 3. Group therapy participation from day 6. Medication dispensed daily under nursing supervision throughout. Suicidal ideation reassessed on days 3, 7, and 12 — progressive reduction documented, complete remission by day 10.

Standardised Assessment Outcomes: PHQ-9 improved from 22 (severe) at admission to 6 (mild) at discharge. GAD-7 improved from 14 to 5. Suicidal ideation subscale: 3 at admission, 0 at discharge. Sleep quality improved substantially with mirtazapine — patient reported 7 hours uninterrupted sleep by discharge versus 3–4 hours on admission.

Ward Activities: Mr. Schröder participated fully in the therapeutic programme from day 5 — group CBT, relaxation, mindfulness, and structured exercise group (which he found particularly helpful). He resumed reading — a valued activity abandoned in preceding months — and began a structured daily activity log as recommended by the psychologist. By discharge he was euthymic, future-oriented, and making concrete plans for a phased return to work.

Risk Assessment at Discharge: Risk of self-harm assessed as low. Key protective factors: strong partner support, re-engagement with daily activities and hobbies, absence of suicidal ideation, preserved insight, concrete future plans. Written safety plan completed and reviewed with patient and partner. Partner given crisis contact details and education on early warning signs.

Medication at Discharge: Sertraline 150 mg once daily in the morning, mirtazapine 30 mg once daily at bedtime.

Follow-up: Outpatient psychiatry in 1 week. Continuation CBT arranged (8 sessions). GP in 2 weeks. Occupational health referral for phased return-to-work planning.

Multidisciplinary Team Review and Discharge Decision:

Mr. Schröder was discussed at the weekly ward MDT meeting on days 7 and 14. The MDT included the responsible consultant, ward registrar, ward psychologist, clinical nurse specialist, social worker, and occupational therapist. On day 7 the team agreed to continue the current plan with the target of achieving further improvement in mood and complete resolution of suicidal ideation before discharge planning. On day 14 the MDT agreed that Mr. Schröder met the criteria for discharge: mood markedly improved, suicidal ideation absent for more than 5 consecutive days, insight preserved, concrete follow-up plan in place, and a supportive home environment confirmed.

Occupational Therapy and Return-to-Work Planning:

The occupational therapist assessed Mr. Schröder on day 10. Functional capacity for all activities of daily living was intact — no home adaptations required. The primary focus was vocational rehabilitation. The OT collaborated with the ward psychologist to develop a phased return-to-work framework: weeks 1–2 at 50% of normal hours with no after-school commitments; weeks 3–4 at 75% hours; full return in weeks 5–6 subject to review. This plan was communicated in writing to the patient's GP for coordination with the school's occupational health service.

Dr. Eva Bergmann, MD

Specialist in Psychiatry and Psychotherapy